BREECH PRESENTATION

DR TANVI VASALLA

- Mrs.x 25 yr old female
- Resident of Nandgaon, vadodra
- Wife of Mr.y shopkeeper by occupation
- Hindu by religion
- Homemaker by occupation
- educated till graduation
- Belongs to upper middle class according to modified kuppuswami scale
- Obstretic score:G2P1L1 with previous normal delivery
- LMP -16/8/2021
- EDD-9/5/2022

- Patient came with 9 months of gestation for safe confinement of pregnancy

Hopi

Patient came for safe confinement.

No h/o pain abdomen

No h/o bleeding or leaking per vaginum

No h/o swelling of legs

Able to perceive fetal movements well

MENSTRUAL HISTORY

Age of menarche 14 yrs

LMP 16/8/2021

EDD 9/5/2022

PAST cycles regular 3-5 days / 28-30 days

No history of dysmenorrhea or passage of clots

Obstetric history

- Score;G2P1L1:
- Active married life 8 years
- P1- 2.8 kg female child age 4 years full term normal delivery at hospital no postpartum complications,baby is alive and healthy
- Child healthy , exclusive breastfeeding for 6 months, fully immunised and normal developemental milestones
- No contraceptive used
- G3-booked and immunised at gov hospital
- 2 doses of TT taken at 3rd and 5th month
- Antenatal visits:12

First trimester

- Spontaneous conception
- Non consanguineous marriage
- LMP -16/8/2021
- EDD-09/5/2022
- NO history of irregular cycles hence sure of date
- Diagnosed at 1.5 months of amenorrhea by UPT
- Booked at government hospital at 2 months of period of gestation
- Advised anc investigations-normal
- Dating scan done: normal
- Advised 1 yellow tablet od taken regularly

No history of excessive nausea and vomiting , fever or rashes, radiation exposure, drug exposure, bleeding per vaginum, discharge per vaginum, burning micturition.

Second trimester:

Quickening at 3 months of amenorrhea

Perceiving adequate fetal movements

2 doses of TT taken at 3 and 5 th month

Vaccinated for COVID as well

second usg done at 5th months-normal

Lab investigations done-normal

History of regular intake of red/yellow/white tablets

No history of high BP Recorded

No history of epigastric pain, blurred vision, headache,decreased urine output and swelling of feet.

No history of excessive hunger and thirst, frequency of micturition or recurrent UTI

No history of easy fatiguability, breathlessness, dyspnea on exertion. Palpitation, syncopal attack and chest pain

No history of fever, burning micturition, bleeding per vaginum and leaking per vaginum

Third trimester:

- Fetal movements well perceived\no history or bleeding/draining per vaginum
- Iron folic acid and iron tablets taken regularly
- Usg done at 7th and 9th months and was normal

- Past history: not significant
- Family history: nad

- Personal history: adequate sleep
- Normal bowel and bladder
- No history of any addiction/alcohol /smoking
- Belongs to middle class

- Dietary history:

Mixed diet by 24 hr recall method

Consumes 2500 kcal /day

With 62 gms protein daily

GENERAL examination:

- Sitting comfortably
- Concious oriented with time, person and place
- Average build/ well nourished
- General condition fair
- Ht 152 cm
- Bmi-34.6
- Pre pregnancy weight 72 kgs
- Present wt 80 kgs

- Hydration adequate
- Afebrile on touch
- PR 84 /min in rt radial artery with good volume normal character no radiofemoral delay
- all peripheral pulses are palpable.
- BP 160/110 mm hg in right arm in supine position repeated after 15 mins similar finding in left arm
- Mean arterial pressure:78
- RR 18/min thoracoabdominal
- Jvp not raised

- Normal hair texture
- Orodental hygiene fair
- No pallor icterus .cyanosis, clubbing
- No pedal odema
- No palpable lymph node

- Bilateral breast normal

Systemic examination

- Respiratory system: air entry bilateral equal . No added sound
- Cardiovascular system: apex beat in 4th intercostal place in anterior axillary line S1 S2 normal no murmur heard
- Central nervous system:no sensory /motor deficit, all reflxes are normal

ABDOMINAL EXAMINATION

Inspection:

Uniformly distended

Uterine ovoid in longitudinal axis

All quadrants moving well with respiration

Umblicus central/inverted

Linea nigra stria gravidarum seen

No dilated veins

PALPATION:All the inspected findings are confirmed

Local temperature not raised

Sfh=33 cm AG 92 cm

Uterus measures 38 weeks size. Relaxed with fullness of flanks seen

Fundal grip: hard, round independently ballotable mass occupying Upper pole of uterus s/o fetal head

Umblical grip : left side: smooth curved uniform resistance felt s/o spine

right side:multiple small nodule like structures felt s/o limb buds

First pelvic grip:: broad soft bulky not independently ballotable part felt s/obreech occupying the lower pole of uterus.

Second pelvic grip : convergence suggests head is not engaged

AUSCULTATION:
FHS on right side above the umblical regular 142 beats per minute
Per speculum - no leaking
Per vaginum –cervix posterior
multiparous os
cervix undilated uneffaced

Provisional diagnosis

- A booked case of 24 yr female from nandgaon belonging to low socioeconomic status with obstetric score of G2p1l1 with 38 weeks period of gestation having a singleton live intrauterine pregnancy in breech presentation with good fetal heart sounds has been admitted for safe confinement of pregnancy not in labour.